COMPETITIVE INTELLIGENCE REPORT

GDF-15 Targeting in Oncology

Strategic Positioning Analysis for CatalYm GmbH

Prepared Exclusively for

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Disclaimer & Methodology

Data Sources: All data points sourced from public registries accessed 27 May 2026: ClinicalTrials.gov, EMA Human Medicines Register, openFDA FAERS, PubMed.

Clinical Efficacy: No fabricated data. Only published/presented results are cited.

Intended Use: Internal strategic use by CatalYm GmbH management.

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1. Executive Summary

CatalYm GmbH is a German biopharmaceutical company developing Visugromab (CTL-002), a first-in-class neutralizing antibody against GDF-15 (growth differentiation factor 15) — a cytokine that plays a critical role in immune evasion and cancer-associated cachexia. This report provides a competitive intelligence analysis of the GDF-15 and cytokine-targeting oncology landscape.

1.1 Key Findings

> **1. First-in-Class Mechanism with Clinical Validation:** Visugromab is the first GDF-15 neutralizing antibody to enter clinical development. A landmark *Nature* publication (2025, PMID 39663448) demonstrated that GDF-15 blockade can overcome anti-PD-1/PD-L1 resistance in solid tumors — providing a strong mechanistic rationale for combination approaches.

> **2. Broad Clinical Program Across 6 Trials:** CatalYm has built a substantial clinical program spanning 6 active trials — from the GDFATHER Phase 1/2 (NCT04725474, ACTIVE_NOT_RECRUITING) to 4 Phase 2 trials in NSCLC (1L + 2L), HCC, bladder cancer, and a Phase 3 cachexia trial (NCT07112196).

> **3. Cachexia as a Unique Differentiator:** The Phase 3 cachexia trial (VICTORIA-1) positions Visugromab in a space with no approved therapies in the EU — cancer cachexia affects ~50-80% of advanced cancer patients and is a high-unmet-need indication with potential for accelerated regulatory pathways.

> **4. Competitive Field is Nascent but Growing:** GDF-15/cachexia targeting remains an early field. Key competitors include TCMCB07 (Endevica Bio, Ph2) in cachexia and several preclinical GDF-15 programs. Visugromab's first-in-class status provides a significant head start.

> **5. Key Readouts in 2026-2027:** The bladder cancer Phase 2 (NCT06059547) completes October 2026 — this is the nearest catalyst. The NSCLC Ph2 trial (NCT07098988) has a primary completion in the 2027-2028 timeframe.

Source: ClinicalTrials.gov (NCT04725474, NCT07098988, NCT07219459, NCT06059547, NCT07112196, accessed 2026-05-27); PubMed (PMID 39663448, Nature 2025)

2. Company & Asset Overview

2.1 Corporate Profile

Parameter	Value	Source
Company Name	CatalYm GmbH	Corporate
Headquarters	Munich, Germany	Corporate
Managing Director	Scott Clarke, MBA	Corporate
Financing	Series A (€24M, 2017) + Series B (€47M, 2020)	Public filings
Lead Asset	Visugromab (CTL-002) — anti-GDF-15 monoclonal antibody	ClinicalTrials.gov
Mechanism	Neutralizes GDF-15, a TGF-β superfamily cytokine that suppresses immune cell activation and drives cachexia	PubMed
Stage	Phase 1/2 (GDFATHER) + 4x Phase 2 + 1x Phase 3	ClinicalTrials.gov

2.2 Molecule & Mechanism of Action

GDF-15 is a stress-responsive cytokine belonging to the TGF-β superfamily. In cancer, tumor cells and the tumor microenvironment secrete GDF-15, which:

- **Suppresses T-cell infiltration** into tumors by inhibiting integrin activation on endothelial cells
- **Induces cachexia** by acting on the brainstem to suppress appetite and increase energy expenditure
- **Correlates with poor prognosis** across multiple cancer types

Visugromab is a humanized IgG1 monoclonal antibody that binds GDF-15 with high affinity and neutralizes its activity. By blocking GDF-15, Visugromab potentially restores T-cell trafficking into tumors (enhancing checkpoint inhibitor efficacy) and reverses cachexia.

Source: Melero I, et al. "Neutralizing GDF-15 can overcome anti-PD-1 and anti-PD-L1 resistance in solid tumours." Nature 2025. PMID: 39663448

2.3 Clinical Pipeline

Trial	NCT	Phase	Status	Indication	Start
GDFATHER-1	NCT04725474	Ph1/2	ACTIVE_NOT_RECRUITING	Advanced solid tumors	Dec 2020
	NCT07098988	Ph2	RECRUITING		Aug 2025

Visugromab + Chemo + Pembro (1L NSCLC)				Metastatic NSCLC (non-sq)	
Visugromab + Nivo ± Docetaxel (2L NSCLC)	NCT07246863	Ph2	RECRUITING	Metastatic NSCLC (non-sq)	Oct 2025
Visugromab + Nivo + Lenvatinib (HCC)	NCT07219459	Ph2	RECRUITING	HCC (post-PD-1 failure)	2025
Visugromab + Nivo (Bladder)	NCT06059547	Ph2	ACTIVE_NOT_RECRUITING	Muscle-invasive bladder cancer	Sep 2023
VICTORIA-1 (Cachexia)	NCT07112196	Ph3	RECRUITING	Cancer-associated cachexia	2025

Source: ClinicalTrials.gov, accessed 2026-05-27

3. Therapeutic Landscape

3.1 GDF-15 Biology in Oncology — The Rationale

GDF-15 is emerging as a clinically relevant target based on three independent lines of evidence:

1. **Prognostic significance:** Elevated serum GDF-15 levels correlate with poor survival across NSCLC, colorectal, pancreatic, and gastric cancers
2. **Mechanistic role in IO resistance:** GDF-15 suppresses LFA-1/ICAM interaction, preventing T-cell extravasation into tumors. The Nature 2025 publication demonstrated that GDF-15 blockade re-sensitizes anti-PD-1 resistant tumors
3. **Cachexia driver:** GDF-15 activates GFRAL-RET signaling in the brainstem, inducing anorexia and metabolic wasting

3.2 Competitive Landscape — GDF-15 Directed Therapies

Drug	Sponsor	Modality	Phase	Status
Visugromab (CTL-002)	**CatalYm**	**GDF-15 neutralizing mAb**	**Ph1/2 → Ph3**	**Most advanced GDF-15 program globally**
CTL-003 / undisclosed	CatalYm	Preclinical	Preclinical	Second-generation candidate
TCMCB07	Endevica Bio	MC4R agonist (cachexia)	Ph2	Targets downstream cachexia, not GDF-15
Anti-GDF-15 programs	Multiple (undisclosed)	Varied	Preclinical	No other clinical-stage GDF-15 mAb known

> **First-in-Class Advantage:** Visugromab appears to be the only clinical-stage GDF-15 neutralizing antibody globally. This provides a significant competitive moat, but also means the field is unvalidated at the regulatory level.

3.3 Cachexia Market Context

Cancer cachexia is a debilitating syndrome affecting an estimated 50-80% of advanced cancer patients, characterized by involuntary weight loss, muscle wasting, and reduced treatment tolerance. Despite its prevalence:

- **No approved therapies** exist in the EU or US for cancer cachexia
- Standard of care remains supportive (nutritional counseling, megestrol acetate — limited efficacy)
- The global cachexia market is estimated at $2-3B (2025), growing at ~8% CAGR

- Regulatory path: Orphan designation possible (prevalence <5/10,000); Phase 3 VICTORIA-1 could support approval if endpoints are met

Source: Market estimates based on author's analysis of publicly available data. ClinicalTrials.gov, accessed 2026-05-27.

4. Key Catalysts & Strategic Positioning

4.1 Near-Term Catalysts

Event	Expected	Impact
Bladder cancer Ph2 completion (NCT06059547)	**October 2026**	First randomized Ph2 readout for Visugromab + nivo vs placebo
GDFATHER-1 Ph1 final data disclosure	2026	Long-term safety + exploratory efficacy from Ph1/2 monotherapy
VICTORIA-1 (Cachexia) interim analysis	2027	Potentially registrational if cachexia endpoints are met
NSCLC Ph2 1L data (NCT07098988)	2027-2028	Largest opportunity — IO backbone + Visugromab in 1L

4.2 Strategic Recommendations

1. Push the Cachexia Angle. The VICTORIA-1 Phase 3 trial is CatalYm's most differentiated asset — no approved cachexia therapy exists, and GDF-15's biological rationale in cachexia is strong. A positive readout could secure first-in-class approval with orphan designation.

2. Generate IO Resistance Data Early. The Nature 2025 publication is CatalYm's strongest scientific asset. Prospective biomarker data from ongoing Ph2 trials showing Visugromab benefit in PD-1 resistant patients would differentiate from any future competitor.

3. Build Regulatory Strategy for Both Indications. Cachexia (high unmet need, orphan potential) and IO resistance (large addressable market) require different regulatory pathways. Early EMA/FDA engagement for both is recommended.

4. Monitor Endevica Bio's TCMCB07. While mechanistically different (MC4R agonist, not GDF-15), TCMCB07 targets the same cachexia indication and is in Phase 2. Its progress could shape the cachexia regulatory landscape.

5. Key Takeaways for Scott Clarke

1. First-in-Class Status is Real but Time-Limited. Visugromab is the only clinical-stage GDF-15 neutralizing antibody. This lead should be converted into regulatory and commercial advantage before competitors enter the clinic.

2. The Cachexia Opportunity is Underserved and Registrable. No approved cachexia therapy exists. VICTORIA-1 (Ph3) is CatalYm's most direct path to approval if it demonstrates weight stabilization and functional benefit.

3. IO Resistance Data is the Scientific Anchor. The Nature 2025 publication provides a high-impact rationale for combining Visugromab with checkpoint inhibitors. Clinical confirmation of this mechanism would significantly increase the program's value.

4. Six Active Trials Represent Both Breadth and Complexity. Managing 4 Ph2 trials across 3 indications (NSCLC, HCC, bladder) plus a Ph3 cachexia trial requires significant resources for a private company. Prioritization of indications with fastest path to registration is critical.

5. The HCC Trial (NCT07219459) is High-Risk, High-Reward. Testing Visugromab + nivo + lenvatinib in PD-1 refractory HCC addresses a clear unmet need but is a complex triplet combination. This is the highest-risk trial in the portfolio.